A trimester-by-trimester framework with nutrition targets, food lists, and a weekly check-in system.

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How to Use This System

This system has three parts: **track**, **nourish**, and **adjust**. You track your weight week by week, nourish your body with the right nutrients at the right time, and adjust your intake based on how your pregnancy is progressing — not on a rigid diet.

1. Find your starting point

Use the table on the next page to find your recommended total weight gain based on your pre-pregnancy BMI. This is your target range.

2. Track weekly

Weigh yourself once a week, same day, same time, same scale. Record it in the tracker on page 6. Small fluctuations are normal — look at the trend, not the daily number.

3. Nourish by trimester

Each trimester has different nutritional priorities. Use the nutrition board on page 4 and the meal frameworks on page 5 to guide your choices.

4. Adjust as needed

If you're gaining too fast or too slow, adjust by 100–200 calories per day and recheck in two weeks. If you're consistently outside your range, talk to your clinician.

Important: This system is based on IOM/ACOG guidelines. It is not medical advice. Always consult your own clinician before making changes to your diet or exercise routine.

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Weight Gain by Pre-Pregnancy BMI

The Institute of Medicine (IOM) and ACOG recommend the following total weight gain ranges based on your pre-pregnancy body mass index (BMI):

Pre-Pregnancy BMI	Recommended Total Gain	2nd/3rd Trimester Rate
Underweight (< 18.5)	28–40 lbs (12.5–18 kg)	1–1.3 lbs/week
Normal (18.5–24.9)	25–35 lbs (11.5–16 kg)	0.8–1 lb/week
Overweight (25–29.9)	15–25 lbs (7–11.5 kg)	0.5–0.7 lbs/week
Obese (30+)	11–20 lbs (5–9 kg)	0.4–0.6 lbs/week

Source: Institute of Medicine, *Weight Gain During Pregnancy: Reexamining the Guidelines*; ACOG Committee Opinion 548.

Where Does the Weight Go?

In a typical pregnancy, weight gain is distributed roughly as follows:

Component	Weight
Baby	7–8 lbs (3.2–3.6 kg)
Placenta	1–2 lbs (0.5–0.9 kg)
Amniotic fluid	2 lbs (0.9 kg)
Breast tissue	1–3 lbs (0.5–1.4 kg)
Increased blood volume	3–4 lbs (1.4–1.8 kg)
Increased fluid volume	2–3 lbs (0.9–1.4 kg)
Uterus	2 lbs (0.9 kg)
Maternal fat stores	6–8 lbs (2.7–3.6 kg)

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Nutrition Board — Daily Targets

These are the key nutrients for pregnancy, with food sources and daily targets. Focus on food first; supplements fill gaps, not replace meals.

Nutrient	Daily Target	Key Food Sources
Folate	600 mcg	Lentils, spinach, asparagus, fortified cereals
Iron	27 mg	Lean red meat, lentils, spinach, fortified oats
DHA	200–300 mg	Salmon, sardines, walnuts, chia seeds
Choline	450 mg	Eggs, lean beef, salmon, broccoli
Calcium	1,000 mg	Yogurt, milk, fortified plant milk, cheese
Vitamin D	600 IU	Fortified milk, salmon, egg yolks, sunlight
Protein	71 g	Chicken, eggs, Greek yogurt, lentils, tofu
Fiber	28 g	Oats, berries, beans, whole grains, vegetables

Note: These are general targets. Your clinician may recommend different amounts based on your blood work, dietary restrictions, or pregnancy complications.

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Calorie Needs by Trimester

Trimester	Extra Calories/Day	Focus
1st (weeks 1–13)	0 extra	Manage nausea, stay hydrated, prenatal vitamin
2nd (weeks 14–27)	+300	Protein + iron for baby's growth spurt
3rd (weeks 28–40)	+450	DHA + calcium for brain and bone development

Trimester Meal Frameworks

These are sample day structures — not rigid meal plans. Adjust portions based on your hunger, activity level, and clinician's guidance.

First Trimester (Weeks 1–13)

Focus: managing nausea, staying hydrated, getting folate. Eat small, frequent meals. Cold foods often smell less and are easier to tolerate.

Meal	Example
Morning	Dry toast or crackers before getting up; ginger tea
Breakfast	Fortified oatmeal with banana and walnuts
Snack	Greek yogurt with berries
Lunch	Lentil soup with whole-grain bread
Snack	Apple slices with peanut butter
Dinner	Baked chicken, steamed broccoli, white rice
Evening	Warm milk or fortified plant milk

Second Trimester (Weeks 14–27)

Focus: protein and iron for baby's growth spurt. Nausea usually fades — appetite returns. Add ~300 calories per day.

Meal	Example
Breakfast	Scrambled eggs with spinach, whole-grain toast, orange
Snack	Trail mix: almonds, dried apricots, dark chocolate chips
Lunch	Quinoa bowl with chickpeas, roasted vegetables, tahini
Snack	Hummus with carrot and cucumber sticks
Dinner	Grilled salmon, sweet potato, steamed green beans
Evening	Cottage cheese with pineapple

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Third Trimester (Weeks 28–40)

Focus: DHA for brain development, calcium for bones. Baby is pressing on your stomach — smaller, more frequent meals work better. Add ~450 calories.

Meal	Example
Breakfast	Overnight oats with chia seeds, blueberries, honey
Snack	Hard-boiled egg + whole-grain crackers
Lunch	Turkey and avocado wrap with side salad
Snack	Smoothie: banana, spinach, Greek yogurt, flaxseed
Dinner	Lean beef stir-fry with brown rice and bok choy
Evening	Warm milk with turmeric (golden milk)

Foods to Limit or Avoid

Avoid	Why
Raw or undercooked fish/meat	Risk of listeria, toxoplasmosis, salmonella
High-mercury fish (shark, swordfish, king mackerel)	Mercury harms baby's brain development
Unpasteurized dairy and soft cheeses	Risk of listeria
Deli meats (unless heated to steaming)	Risk of listeria
Alcohol	No safe amount during pregnancy
Excess caffeine (> 200 mg/day)	May increase miscarriage risk
Raw sprouts	Risk of salmonella and E. coli

Weekly Check-In Tracker

Use this tracker every week. Record your weight, how you're feeling, and any adjustments you're making. Look at the trend — not the daily number.

Week	Date	Weight (lbs)	Weekly Gain	Notes
Week 1				
Week 2				
Week 3				
Week 4				
Week 5				
Week 6				
Week 7				
Week 8				
Week 9				
Week 10				
Week 11				
Week 12				

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Check-In Prompts

- How was my energy this week?
- Did I eat protein at every meal?
- Did I take my prenatal vitamin daily?
- How is my digestion? (constipation is common in pregnancy)
- Am I staying hydrated? (aim for 8–10 glasses of water)
- Do I need to adjust my calorie intake based on this week's trend?

When to call your clinician: You are losing weight in the first trimester (beyond normal nausea) · You are gaining weight too quickly or too slowly · You have concerns about your weight or nutrition · You have a history of an eating disorder.

Disclaimer & Sources

Not medical advice. This guide is for informational purposes only. It does not replace professional medical advice, diagnosis, or treatment. Always consult your own clinician. Individual weight gain recommendations vary based on your health, your baby's growth, and your clinician's guidance.

Sources: Institute of Medicine, *Weight Gain During Pregnancy: Reexamining the Guidelines* (2009) · ACOG Committee Opinion 548 (reaffirmed 2020) · NIH Office of Dietary Supplements · USDA Dietary Guidelines for Americans.

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